CLINICAL · EAR

Myringitis

Myringitis is inflammation of the drum itself. In the bullous form, blood-filled blisters on the drum cause sudden, intense pain that settles in a few days. In the granular form, the drum's skin is replaced by wet granulation tissue that discharges without pain, and the middle ear behind it is healthy.

AT A GLANCE

- Bullous: severe pain with blisters on the drum.
- Granular: painless foul discharge with granulation.
- Rule out middle ear disease.
- Treat with analgesia and drops, or vinegar and removal.

1 CHAPTER ONE Overview

Disease of the drum, not the middle ear.

1.1 Definition

Inflammation confined to the tympanic membrane, sometimes with the adjacent deep canal skin, without primary disease of the middle ear or mastoid.

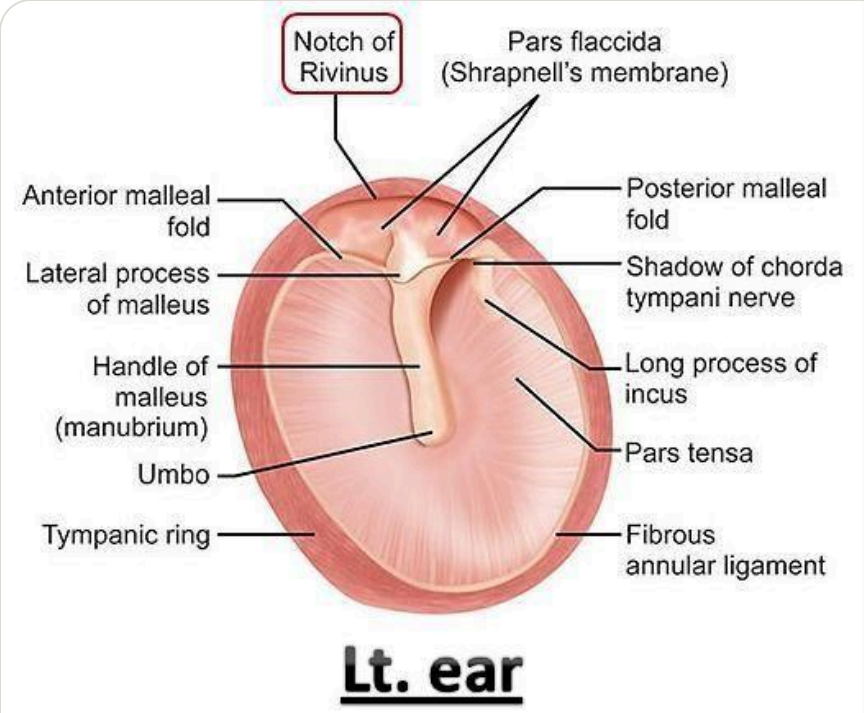


FIGURE 1 Parts of the tympanic membrane.
Anatomy atlas · ENT Steps p. 36

1.2 Forms

1.2.1 Acute bullous myringitis

Blebs or bullae on the outer surface of the drum and the deep canal. Dhingra describes the same picture as *otitis externa haemorrhagica*.

1.2.2 Chronic granular myringitis

The epithelium of the drum is replaced by granulation tissue, without a perforation.

2 CHAPTER TWO Bullous myringitis

Blisters and pain.

2.1 Cause

It follows an upper respiratory infection. The organisms are those of acute otitis media: *Streptococcus pneumoniae*, *Haemophilus influenzae* and *Moraxella*. *Mycoplasma*

is the classic association, but it has not been proven.

WHERE THE BOOKS DIFFER · MYCOPLASMA

Dhingra	Lee
Lists it as a possible cause.	The link is unproven.

2.2 Clinical picture

Sudden, intense ear pain for three or four days that then settles, sometimes with a little bloody discharge when the blebs burst. Haemorrhagic bullae sit on the drum and deep canal. A third to two-fifths have a middle ear effusion as well.

2.3 Hearing

A sensorineural or mixed loss, often in the high frequencies, is common and usually recovers. Always check hearing.

2.4 Treatment

Analgesia; lancing the blebs relieves pain; keep the ear dry; topical drops to prevent secondary infection; treat acute otitis media if present.

3 CHAPTER THREE Granular myringitis

A wet, painless drum.

3.1 Cause

Usually *Staphylococcus*, *Pseudomonas* or *Proteus*.

WHERE THE BOOKS DIFFER · PREDISPOSING FACTORS

Dhingra	Lee
Links it to wax, foreign bodies and external ear infection.	No predisposing factor has been identified.

3.2 Clinical picture

Itching, fullness and a foul, painless discharge. Granulation tissue covers part of the drum, often posterosuperiorly; the drum may perforate and heal repeatedly. Hearing is normal or slightly reduced.

3.3 Diagnosis

Clinical, under the microscope. CT only if chronic otitis media or cholesteatoma has to be excluded.

3.4 Treatment

Regular cleaning, dilute vinegar irrigation and topical drops; cauterising the granulation with silver nitrate or trichloroacetic acid, or laser; removal of the granulation. Severe or recurrent cases need tympanoplasty with canalplasty. Recurrence is common after surgery.

3.5 Complication

Late fibrosis can blunt the anterior angle between the drum and canal wall, causing a conductive loss.

4 CHAPTER FOUR Differential diagnosis

Other painful or wet drums.

- **Acute otitis media:** bulging drum with poor mobility.
- **Chronic otitis media:** a perforation and middle ear disease.

- **Herpes zoster oticus:** vesicles on the concha and canal, facial palsy.
- **Otitis externa:** canal swelling and tragal tenderness.

5 CHAPTER FIVE For the examination

What the examiner wants to hear.

5.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- **Bullous versus granular,** and the symptoms of each.
- **Sensorineural loss** in bullous myringitis.
- **Treatment** of each form.

5.2 Common pitfalls

COMMON PITFALLS

- Not checking hearing in bullous myringitis.
- Calling granular myringitis chronic otitis media without examining under the microscope.

Written for this app from Oxford Textbook of Otolaryngology ch. 13, Oxford Specialist Handbook ch. 15, Dhingra ch. 8 and K. J. Lee ch. 22. Figures reproduced from the books for private study. Study reference only: follow local protocols and senior advice for patients.